ONERHYTHM

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PROJECT ONE RHYTHM

Project Brief

The Visibility Arm of a Problem Too Large to Ignore

Prepared by OneRhythm

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> *“The bear you can’t see is the one that does the most damage. Not because it’s stronger. Because nobody believes you when you tell them it’s there.”*

THE HEART

What OneRhythm Is

OneRhythm is community platform for people whose lives have been shaped by cardiac arrhythmia. It is the visibility arm of a broader effort to close a treatment gap that has persisted for decades - the gap between the patients who suffer psychologically and the healthcare systems that have not yet learned to see them.

The platform does not diagnose. It does not treat. It does not analyze ECG data or monitor vital signs. What it does is something that no clinical tool currently does at all: it makes the invisible weight of living with an irregular heartbeat visible, countable, and impossible to ignore.

OneRhythm exists because invisibility is a precondition for neglect. When a population’s suffering cannot be seen, it cannot be prioritized. When it cannot be prioritized, it does not get funded. When it does not get funded, it does not get treated. That chain breaks when someone makes the invisible undeniable.

THE INVISIBLE BEAR

The Problem We Make Visible

There is a particular kind of suffering that comes with cardiac arrhythmia that the healthcare system has not yet learned to name. It is not the arrhythmia itself. It is the fear that accompanies it. The hypervigilance. The dread before sleep. The way a skipped beat can rearrange an entire afternoon. The quiet withdrawal from things that used to matter because the body can no longer be trusted to cooperate.

We call it the invisible bear. It is the psychological burden that walks beside the arrhythmia patient every day—undiagnosed, untreated, and largely unacknowledged by the institutions designed to help. The bear is real. It is measurable. And it is being carried by more than 100 million people worldwide.

The numbers are not ambiguous:

88.3% of arrhythmia patients experience anxiety • 71.1% experience depression • 20% report suicidal ideation

Patients with implantable defibrillators face a 3.92-fold increase in clinical anxiety with each shock event. Cardiac PTSD carries a 3.2-fold increase in five-year mortality. Patients with premature ventricular contractions - often dismissed as “benign” by their physicians - report anxiety and depression rates of 40-60%. The global atrial fibrillation population alone exceeds 52.55 million as of 2021data, and the lifetime risk of developing any arrhythmia is one in three.

Yet of the 15-40% who screen positive for clinical anxiety or depression on validated instruments, only 2.5% are identified through routine clinical screening. The care delivery gap is not a screening failure. It is a visibility failure. The problem is not that the tools to detect distress do not exist. The problem is that the distress itself is not visible enough to demand action.

THE EVIDENCE

Why Visibility Matters Now

In 2025, the European Society of Cardiology published its first-ever Clinical Consensus Statement on Mental Health and Cardiovascular Disease. For the first time, a major cardiology body formally recommended systematic mental health screening during cardiovascular care and called for the creation of multidisciplinary Psycho-Cardio Teams. The American Heart Association issued parallel recommendations. The ACTIVE framework - Acknowledge, Check, Tools, Implement, Venture, Evaluate - was introduced as the operational model for integrated care.

The institutional mandate now exists. What does not yet exist is the public infrastructure to support it. Psycho-Cardio Teams cannot be built without awareness. Screening cannot be prioritized without demand. Funding cannot flow to a problem that the public and its representatives cannot see.

This is the gap OneRhythm fills. Not by treating. Not by diagnosing. By making the scale of the crisis undeniable. By turning individual stories into collective data. By creating a public artifact - visible, measurable, growing - that makes it impossible to look at the intersection of arrhythmia and psychological distress and say: this is not a real problem.

Särnholm et al. demonstrated that when cardiac-related fear and avoidance are treated with targeted cognitive behavioral therapy, healthcare consumption drops by 56% - sustained at 12 months. The mechanism is not fixing the rhythm. It is addressing the psychological response to the rhythm. That distinction matters enormously. It means that a large share of the clinical burden is not cardiac. It is *psychological*. And it is treatable - if anyone is looking for it.

THE MISSION

Shared Rhythm Distance

Every contribution to OneRhythm advances a shared public mission by exactly 0.75 meters—a symbolic 30-second rhythm strip at standard ECG recording speed of 25 mm/s. The metaphor is intentional. It is the length of time a wearable device needs to capture a single heartbeat story. It is the smallest meaningful unit of participation. And when multiplied by millions of people, it becomes something large enough to be seen from space.

The mission route begins in Tampa, Florida. It circles the Earth. It returns to Cape Canaveral. And it continues to the Moon. The Earth’s circumference is approximately 40,075 kilometers. That requires roughly 53.4 million contributions. The Moon is 384,400 kilometers away. That requires roughly 512.5 million.

If the global AF population of 52.55 million people each contributed one symbolic waveform, the shared rhythm would reach 98.3% of the way around the Earth.

The math is the message. The problem is large enough that the people who carry it could wrap a signal around the planet. And nobody has asked them to do it until now.

The Mission to the Moon visualization is a real-time 3D experience rendered with a NASA-style dashboard showing the mission route, country-level participation data, a live contribution feed, and a rhythm chain of recently generated art tiles scrolling across the bottom of the screen. The Moon is visible in the scene. The distance is real. The contributions are real. The only thing that is not real is the idea that this population does not matter.

THE TRIFECTA

Heart, Mind, Hands

There is a saying that has guided this work from the beginning: heart, mind, hands. They are all you ever really need.

OneRhythm is the heart. It is the visibility arm. The emotional truth. The platform that says: you are not alone, your experience is real, and the world is going to see it. OneRhythm does not solve the clinical problem directly. It creates the conditions under which the clinical problem becomes impossible to ignore. Visibility precedes funding. Funding precedes infrastructure. Infrastructure precedes treatment. The heart comes first.

Project MIRmade is the hands. Model Irregular Rhythms - Made with ❤. It is the clinical research initiative that takes the problem OneRhythm makes visible and builds the AI/ML systems to do something about it. MIRmade assembles ECG foundation models, clinical NLP, wearable biosignal analysis, and digital phenotyping to predict psychological distress in arrhythmia patients before crisis - closing the gap between the 15-40% who screen positive and the 2.5% who receive treatment. The hands do the work.

Project De Novo is the mind. Built anew. It is the operating architecture that allows one founder to pursue a problem this large without losing coherence, rigor, or truth. De Novo treats memory, scope discipline, drift mitigation, and structured review as first-class infrastructure. It is the recognition that personal medical problems, when translated to population scale, require a different kind of thinking - abstract, disciplined, and unrelenting. The mind holds it all together.

These three initiatives are not separate projects. They are one effort with three dimensions. The heart makes the problem visible. The hands build the tools to address it. The mind ensures the work compounds rather than fragments. Together, they represent a complete response to a crisis that has been ignored for too long.

THE INVITATION

What We Ask

OneRhythm is open source. The platform is positioned as a general wellness and educational resource under FDA enforcement discretion. It is built to be trustworthy from the ground up, because the community it serves has already spent enough time not being believed.

What we ask is straightforward. We ask for partnership. We ask for visibility. We ask for the attention of foundations, research institutions, patient advocacy organizations, electrophysiology societies, and anyone who believes that 88.3% anxiety, 71.1% depression, and 20% suicidal ideation in a population of more than 100 million people is a problem worth solving.

We ask for the chance to make the invisible undeniable.

The technology exists. The evidence is peer-reviewed. The institutional mandate is published. The patient population is enormous and underserved. What has been missing is a platform that gives that population a voice - not a clinical voice, but a human one. A way to say: I am here. This is real. I am not alone.

OneRhythm is that platform.

AUTHOR

Author

Matthew J. Adams is the founder of OneRhythm. As a patient for ten years dealing with high burdens of virtually every kind of arrhythmia, the founding of OneRhythm was not a career decision. It was a consequence of lived experience - redirected toward a problem larger than one patient story.

OneRhythm is the heart. Project MIRmade is the hands. Project De Novo is the mind. Together, they represent an attempt to make a crisis visible, build the tools to address it, and maintain the discipline required to see it through.

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Nobody should fight an invisible bear alone.

OneRhythm makes the bear visible.

#OneRhythm #InvisibleBears #SharedRhythm

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